جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : اسماء شوقي عبدالسلام عيسى
المحافظة : المنوفيه **الحي / المدينة :** اشمون **العنوان :** ش صلاح الدين
رقم البطاقة : 28401101701085 **رقم التليفون :** 01555002010 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 10-01-1984 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه **الإخصائي الاجتماعي**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اسماء شوقي عبدالسلام عيسى
National ID: 28401101701085

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



اسماء
شوقى عبدالسلام عيسى
ش صلاح الدين-بجوار مدرسة المساعى
مركز اشمون - المنوفية

٢٨٤٠١١٠١٧٠١٠٨٥

١٩٨٤/٠١

LC1967251

٢٨٥٠١٠١٧٠١٠٨٥ ٢٠٢٥/٠٤

حاصله على دبلوم تجارة

انثى مسلمة متزوجة

مجدى فؤاد عجمى عجمى شعير

١٢ البطاقة سارية حتى ٢٠٣٢/٠٤/١١

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 7 / 12/ 2025 G

Name	[illegible]	MRN	1458/25
		ID	01235559
		Phone	01001798057
Age	44 Years	Marital status	Married
Menopausal status	peri Menopausal	Family history	-ve
NRS SCORE		REASON OF MDT	post operative
BMI			
CO morbidity	-ve		

Clinical Data

BX: IDC G II

ER:	PR:	HER 2:	Ki67:
7/8	6/8	4+ve	35%

Sonomammography:

post NACT

L+S.M+2LNB

Mets Workup:

→ Free Margins

→ Nipple areola free

→ No residual

R L

T N M

c ubi

Recommendations:

Very satisfactory response

Pathological complete response

O/S.

ypTO / ypNo

MDT OUTCOME & PLAN

Arabi phase upto 17cycle + PORH

+ RT x 25 fx in 2 sides

+ Zometa

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: اسماء شوقي عبدالسلام عيسي MR. : 3071	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient underwent postoperative therapy with FEMARA and Zoladex 10.8 mg administered for the first time. Zometa was to be considered after a dental evaluation, and a PHESGO regimen was planned for the eighth cycle with placement of a port pending.

Zoladex was scheduled for administration on 13 March 2026, and FEMARA was given on the same day. The patient also received PHESGO for three months, with an echo performed to assess response.

For cycle 10, the patient received PHESGO in combination with Zoladex 10.8 mg. The pharmacy recorded administration of cycles 11 and 12, and an echo performed on 8 March 2026 showed a tumor reduction to 68 %.

The patient's left breast cancer was staged as cT4 d, N1, M0, IDC grade 2, ER 7/8, PR 6/8, HER2 23+. The case was discussed on 17 November 2025, and the patient had left sentinel node surgery with 0/5 nodes positive. New laboratory work was ordered; FEMARA was continued, but PHESGO was not available at that time.

During a pharmacy visit, the patient received cycle 12 of PHESGO (600 mg) together with Letrozole. An echo performed on 8 March 2026 again demonstrated a 68 % response.

For cycle 14, the patient was treated with PHESGO, FEMARA, and supportive therapy; an echo on 10 August 2026 showed a 64 % response. Plans were made for cycles 15 and 16, with possible continuation to cycle 17 pending pharmacy approvals, and FEMARA was to be continued.

The most recent pharmacy visit documented administration of cycle 15 of PHESGO together with FEMARA, and an echo on 10 August 2026 showed a 64 % response. The patient has now completed fifteen cycles of PHESGO.

تحريرا في ,

التاريخ :	18-09-2026	الوقت :	

الطبيب المعالج : ندى حسن صلاح كامل

يعتمد ,

atient Name	:الأستاذه / اسماء شوقي عبد السلام عيسي	Request No.	: 13003457
atient No.	:28401101701085	Request Date	: 17/11/2025
ith No.	:3544/25	Verification Date	:27/11/2025
ex	:Female	Financial Category	:جوستاف رومي - نفقة الدولة
ge	:41 Y	Department	: Out Patient
eferreing Doc :		Registered by	:Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer, Post-NACT.
Nature of Specimen: Left simple mastectomy & Sentinel lymph nodes biopsy.

Gross Examination:
* Left simple mastectomy specimen measured 22x21x4 cm, with covering skin ellipse measured 21x13 cm. Nipple & areola looked unremarkable grossly.
* Serialling showed a tumor bed site 8x5x2 cm, located 2 cm from the least deep margin.
* Sentinel lymph nodes: Fibrofatty tissue piece measured 4x3x1 cm, entangling 5 lymph nodes, the largest measured 1.5x1x0.5 cm.
* 11 Blocks:
1:8- T.B.S., 9- N&A., 10- Deep margin., 11- SLNs.
- Gross was done by; Dr. Heba Hassan.

** Frozen (intra-operative) results:
- Free five sentinel lymph nodes (0/5).

Microscopic Examination:
* Histological type: No residual viable remnants of the previously diagnosed invasive tumor.
- Cellularity (0%).
* Angioinvasion: Absent.
* Ductal carcinoma in situ: Absent.
* Additional findings: Effect of therapy with microcalcifications.
* Nipple & areola: Free.
* Deep margin: Free.
* Sentinel lymph nodes: Five lymph nodes free of viable metastatic deposits (0/5) (One lymph node showing effect of therapy).
* Histologic response to therapy: Pathological complete response to therapy.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبني رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

JSTAVE/
OUSSY
NATIONAL

Patient Name	الأستاذه / اسماء شوقي عبد السلام عوسي :	Request No.	: 13003457
Patient No.	: 28401101701085	Request Date	: 17/11/2025
Path No.	: 3544/25	Verification Date	: 27/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Diagnosis:
Left breast cancer (Post-NACT), Left simple mastectomy & sentinel lymph nodes biopsy:
- No residual viable remnants of the previously diagnosed invasive tumor.
- Ductal carcinoma in situ: Absent.
- Deep Margin: Free.
- Nipple & areola: Free.
- Sentinel lymph nodes: Five lymph nodes with no viable metastatic deposits, LNs (0/5) (One lymph node showing effect of therapy).
- Histologic response to therapy: Pathological complete response to therapy.
- Stage: ypT0, ypN0 (sln).

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

ent Name	: الأستاذه / اسماء شوقي عبد السلام عيسي	Request No.	: 13006765
ent No.	: 28401101701085	Request Date	: 14/04/2025
ı No.	: 1299/25	Verification Date	: 27/04/2025
‹	: Female	Financial Category	: اورام دار السلام - مباشره
e	: 41 Y	Department	: Out Patient
ferreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

nical data
ft breast mass

ɜture of specimen
u cut biopsy, collected at 13/4/2025.

echnique:
Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, 'R, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
IPT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 90% of tumor cells (Allred score 7 /8).
- PR: Moderate nuclear reaction in 60% of tumor cells (Allred score 6 /8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 35% nuclear reaction of tumor cells.

Diagnosis:
Left breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade II with focal DCIS.
IPT:
- ER: Positive (score 7 /8).
- PR: Positive (score 6 /8).
- Her2: Positive (score 3+).
- Ki67: Positive in 35% of tumor cells.

* Scoring system: "ER, PR"

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

tient Name	الأستاذه / اسماء شوقي عبد السلام عيسي	Request No.	13006765
tient No.	28401101701085	Request Date	14/04/2025
th No.	1299/25	Verification Date	27/04/2025
ex	Female	Financial Category	اورام دار السلام - مبادره
ge	41 Y	Department	Out Patient
eferreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

SCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, termediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

eference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining bserved or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of ne invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely erceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane taining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or omplete and circumferential membrane staining that is intense and within < 10% of the invasive tumor ells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor ells.

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :

غرفة رقم (4) – الدور الأول – مبانى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Coroiche - Misr El Qadema - Cairo, Egypt

مركز بهية للاكتشاف المبكر
وعلاج سرطان السيدات

Patient Name:	ASMAA SHAWKY ABDEL SALAM EIS	Gender:	Female
MRN:	20225018885	Ordered By:	Hadeel Ahmed Hany Abd Elmohaimen
DOB:	10/01/1984 Age: 41 Y 4 M 9 D	Order ID:	189773

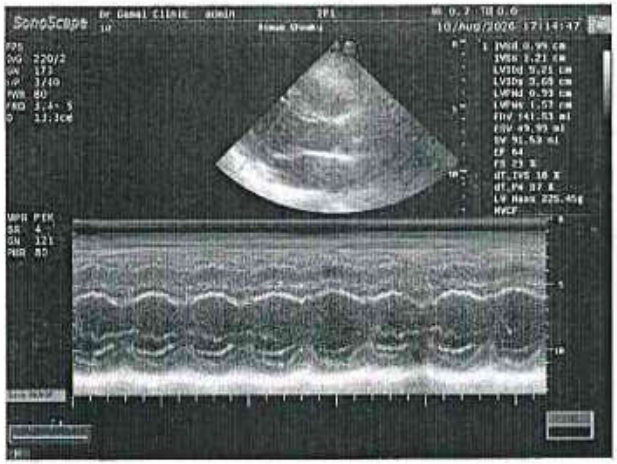
US. Both breast

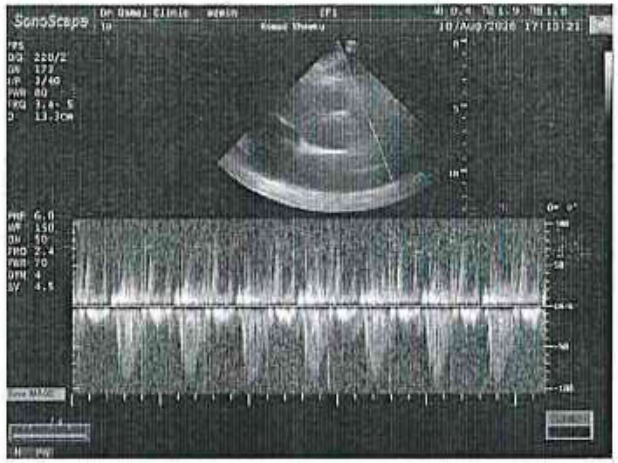
Procedure Date: 08/05/2025 02:47:00 PM

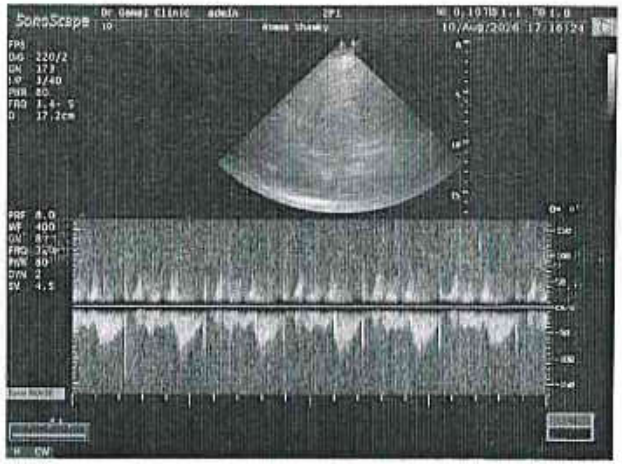
Report:

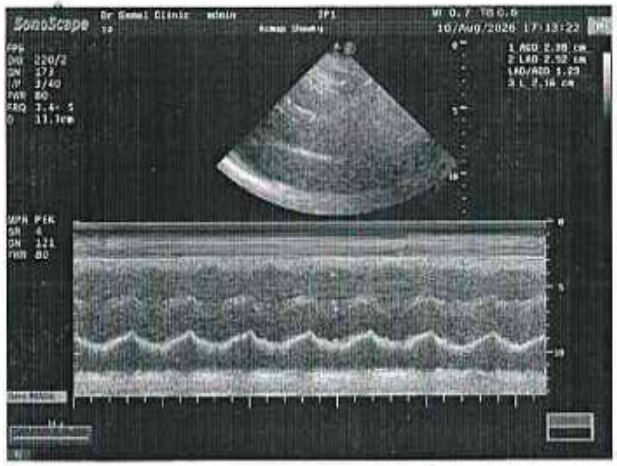
Report:
FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND
INDICATION:
- Left breast pathologically proven outside Baheya IDC grade II with focal DCIS and triple positive.
TECHNIQUE:
- CC and MLO view.
- Complementary ultrasound.
REPORT:
- Extremely dense breasts (ACR D), which can obscure underlying lesions.
Left breast:
- Left LOQ two small groups of suspicious micro calcifications are seen, complementary ultra sound revealed left 3-4 o'clock para areolar focal area of palpable altered parenchyma mounting to mass formation is seen about 0.3 cm from the skin and nearly inseparable from under surface of the nipple. It shows subtle vascularity on applied color Doppler and measures about 5 x 3 cm.
- Left axilla (level I) shows single globular hypo echoic pathological lymph node. It roughly measures about 2.1 x 1.1 cm.
- No pathological supra or infra clavicular lymph nodes.
Right breast:
- No right spiculated mass lesions or grouped microcalcifications.
- Complementary ultrasound revealed no right solid or cystic mass lesions.
- Mottled appearance of prominent fibro glandular parenchyma of the breasts.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right nonspecific axillary lymph nodes with central fatty hila.
- No pathological supra or infra clavicular lymph nodes.
OPINION:
- Left breast pathologically proven malignancy outside Baheya as described (BIRADS 6).
- Left axilla pathological lymph node.
- Fibroadenosis.

Approved by: Ola Aboulnour, MSc
Date:08/05/2025 Time:15:09

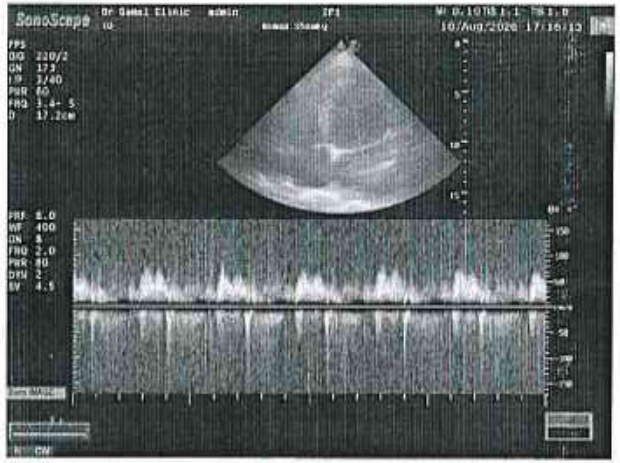

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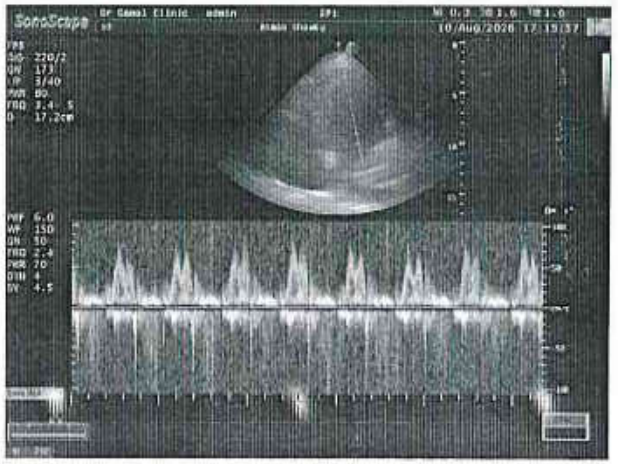

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Echo - Doppler Report

Name : اسماء شوقى عبد السلام **Age :** 40 yrs **Date :** 10/08/2026

Referred by Dr :

	Patient	Normal		Patient	Normal
LVIDD	5.21	3.5-5.6 cm	Aortic root	2.38	2-3.7 cm
LVISD	3.68	2-4 cm	Left Atrium	2.92	1.9-4 cm
Septum	0.99	0.6-1.2 cm	RV. Diameter	2.16	< 3.3 cm
Posterior Wall	0.99	0.6-1.2 cm			
E.F	64	>55	F.S	29	>27 %

Mitral valve : Mild regurge

Aortic valve : Normal

Pulmonary valve : Normal

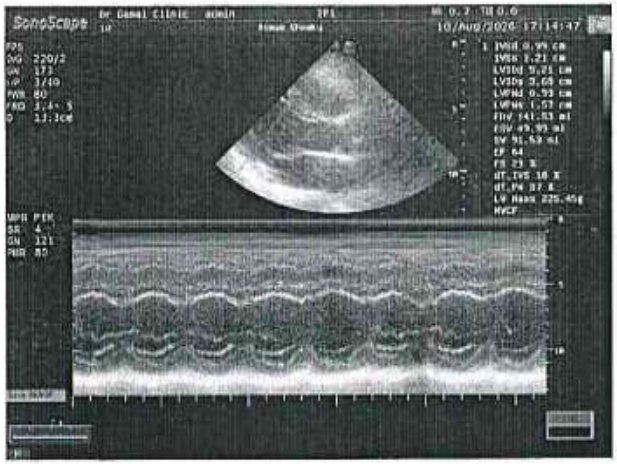
Tricuspid valve : Mild regurge with no evidence of pulmonary hypertension .

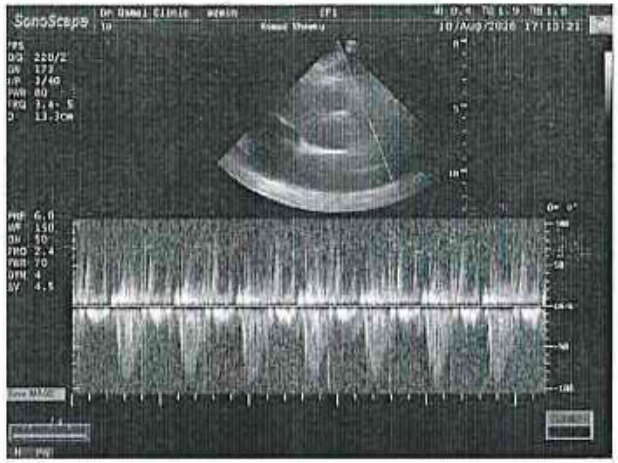
Comment and conclusion :

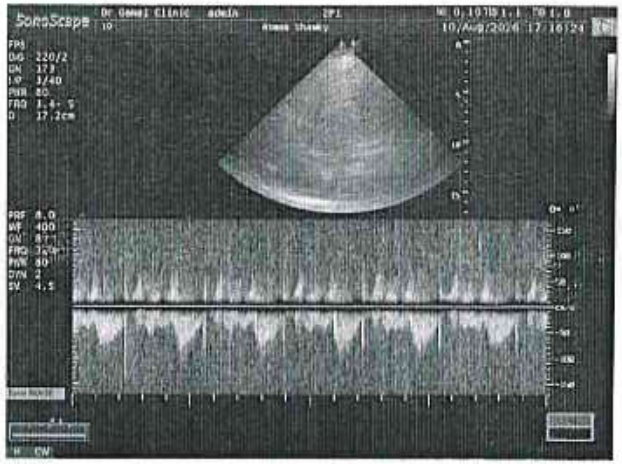
- Normal LV internal dimensions and normal LV systolic function (E.F : 64% by M-mode).
- Normal LA and aortic root dimensions.
- Normal Rt side size and function.
- Mild mitral regurge , mild tricuspid regurge with estimated PASP: 24.90 mmHg and normal other valves.
- No gross wall motion abnormalities at rest could be detected.
- No intracardiac masses or thrombi could be detected.
- Intact interatrial septum and interventricular septum.
- No pericardial effusion.

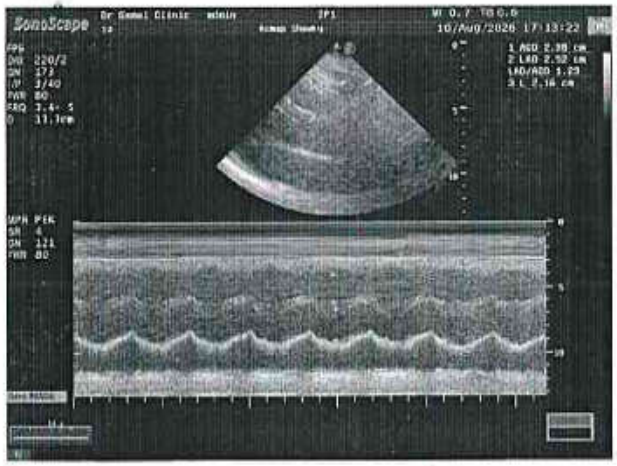
Dr Gamal Dighadi

Cardiologist

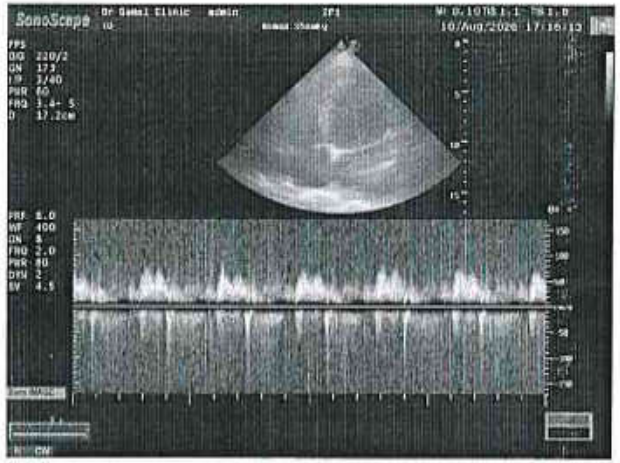

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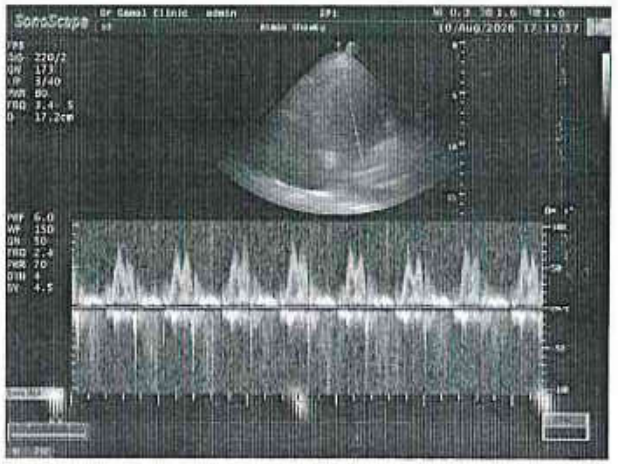

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Echo - Doppler Report

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Mitral valve : Mild regurge

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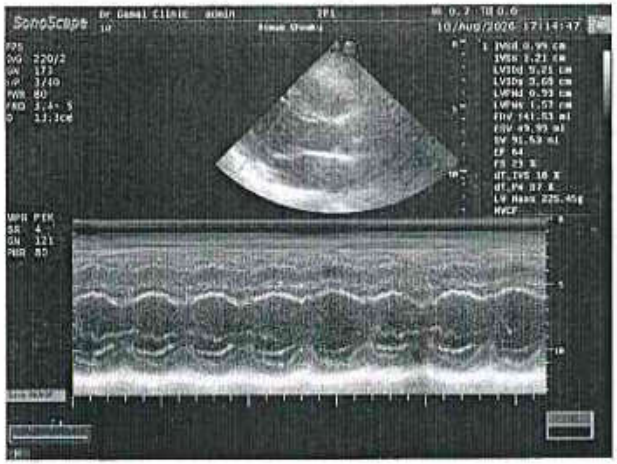
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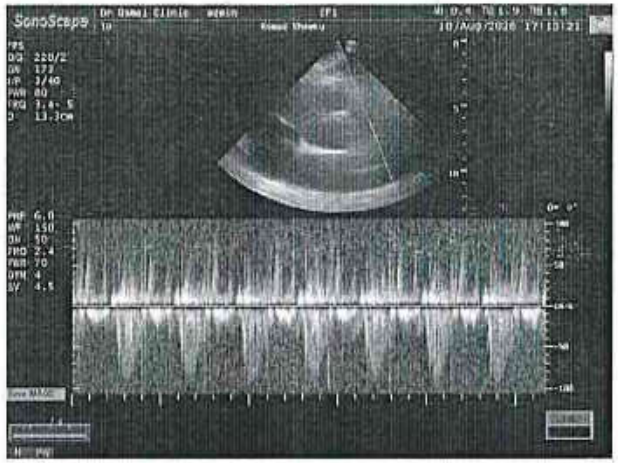
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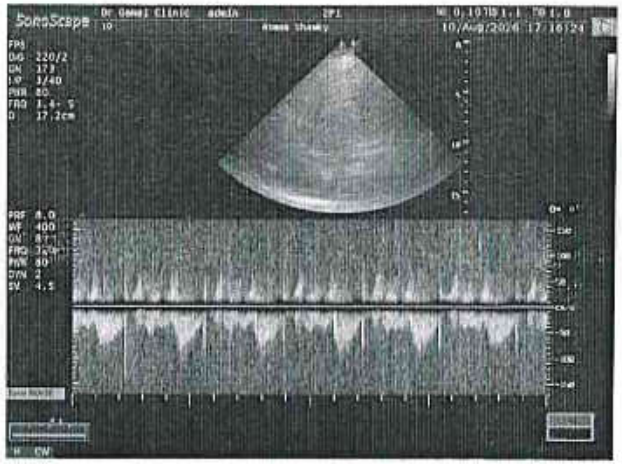
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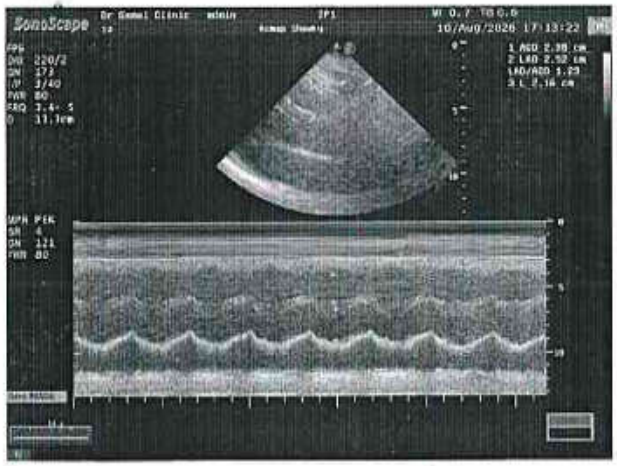
Dr Gamal Dighadi

Cardiologist

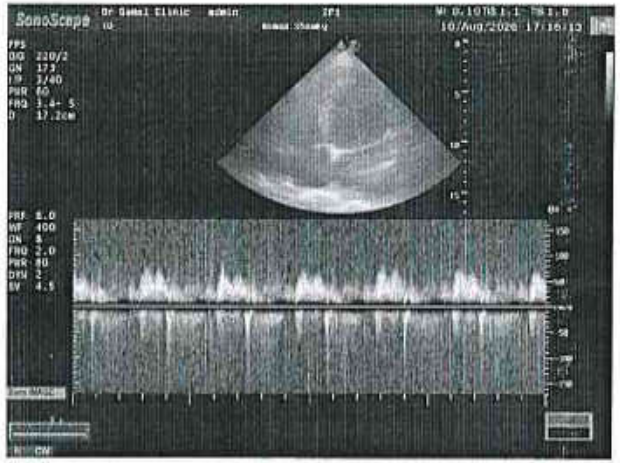

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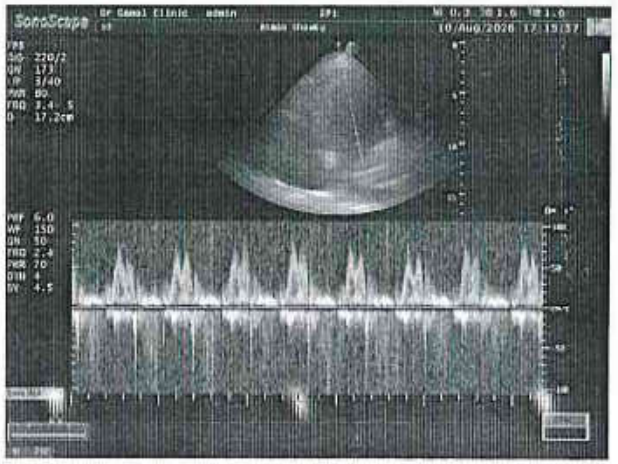

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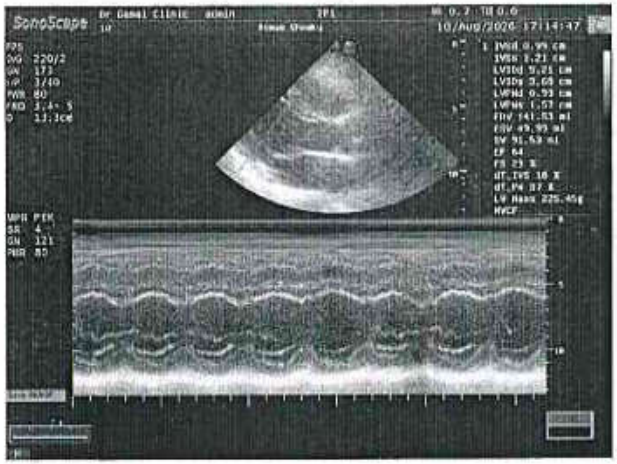
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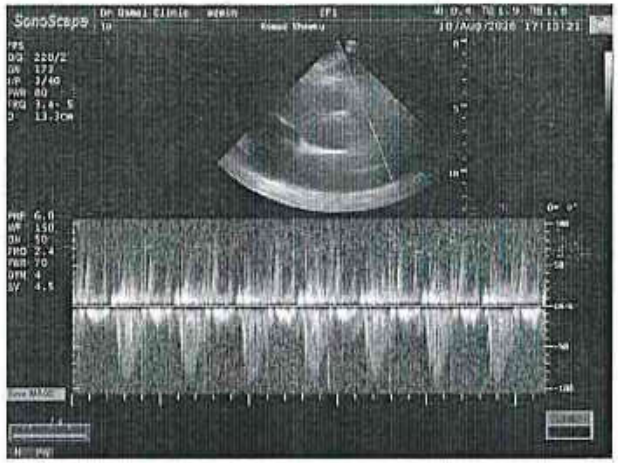
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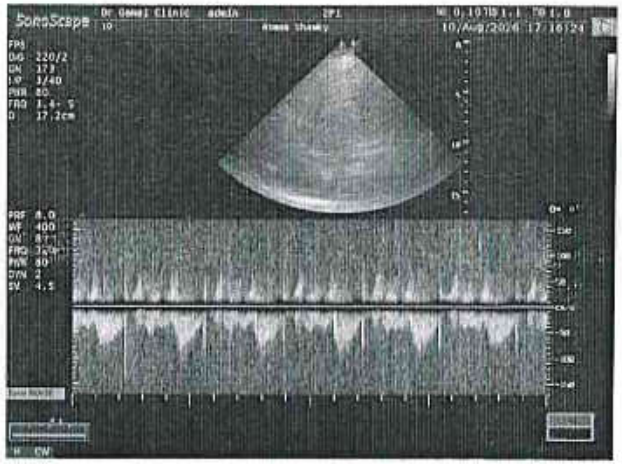
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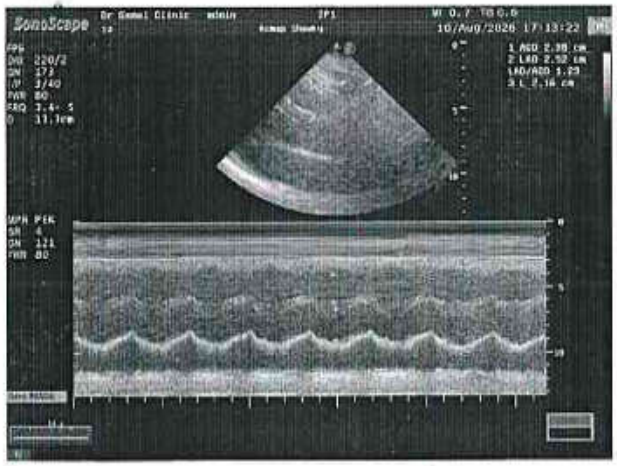
Dr Gamal Dighadi

Cardiologist

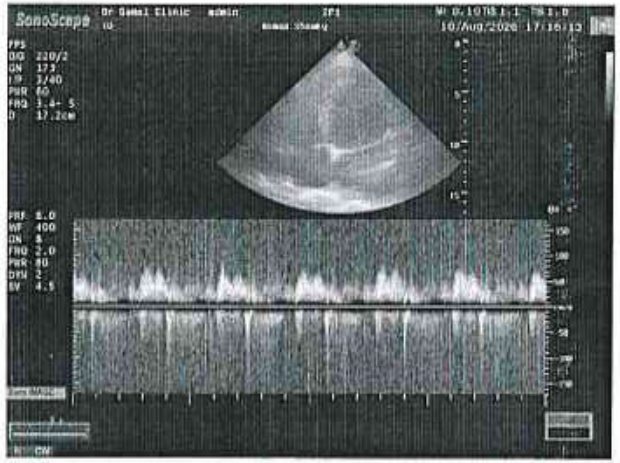

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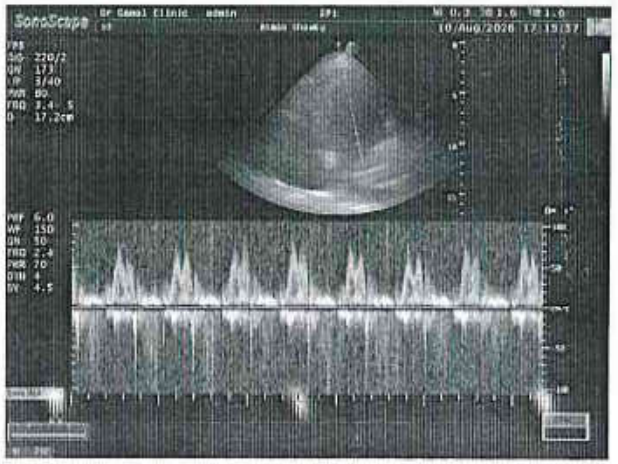

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Dr Gamal Dighadi

Cardiologist

جوستاف روسي
انترناشونال
مصر

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـر طبـي

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشان تقرير طبي للمريض: أ/ سماء شوقي عبد السلام

بقسم.....رقم طبي .....

نتشرف بان نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول.....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

لمريضة تعاني من ورم حميد بالثدي مغلق للعلاج
يوجد لمريضة تلفت ٧ جرعات قبل الجراحة ثم تلفت
جرعة لتاسعة بتاريخ ٢٠٢٥/١٢/١٦ ولتاسعة بتاريخ ٢٠٢٦/١/٩
وجرعة ١٠ بتاريخ ٢٠٢٦/٢/٩٠ ولـ ١١ بتاريخ ٢٠٢٦/٤/١٠
ولـ ١٢ بتاريخ ٢٠٢٦/٧/١١ (لعدم توفر العلاج) ولـ ١٣ بتاريخ
٢٠٢٦/٧/١٤ والـ ١٤ بتاريخ ٢٠٢٦/١٠/١٢ ولـ ١٥
بتاريخ ٢٠٢٦/٩/١٤ وتحتاج الى استكمال العلاج

تحريرا في

الوقت: .....

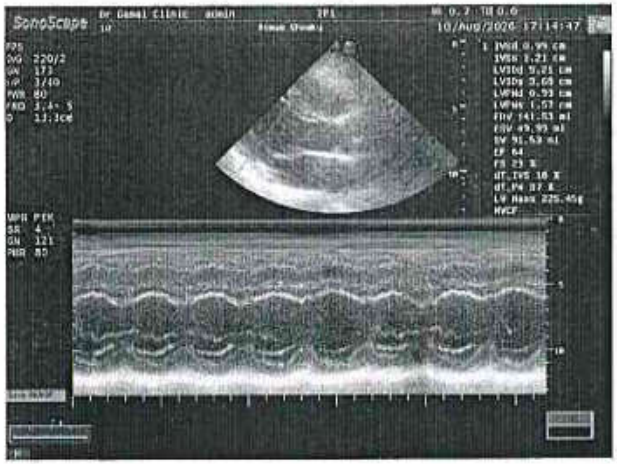
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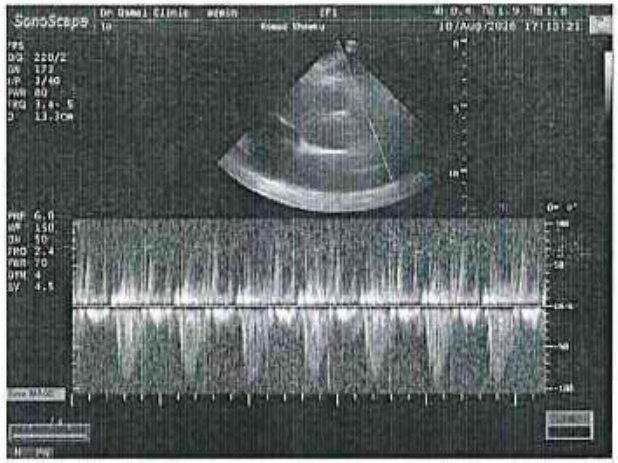
الطبيب المعالج

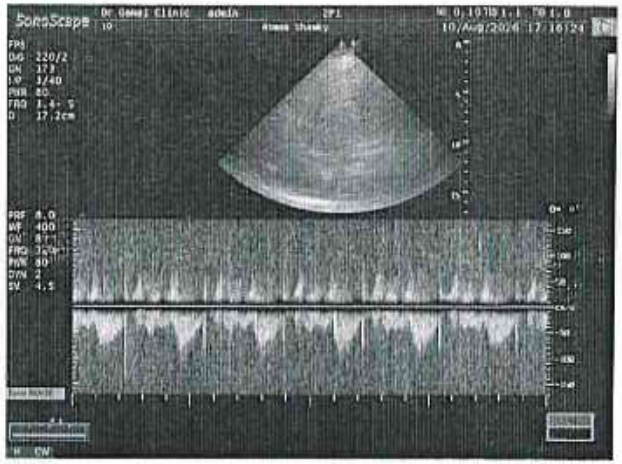
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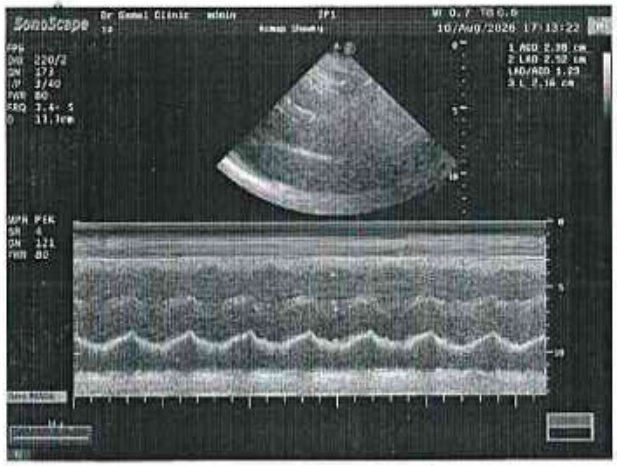
٢٦/٩/٢٠٢٦

يعتمد ،،

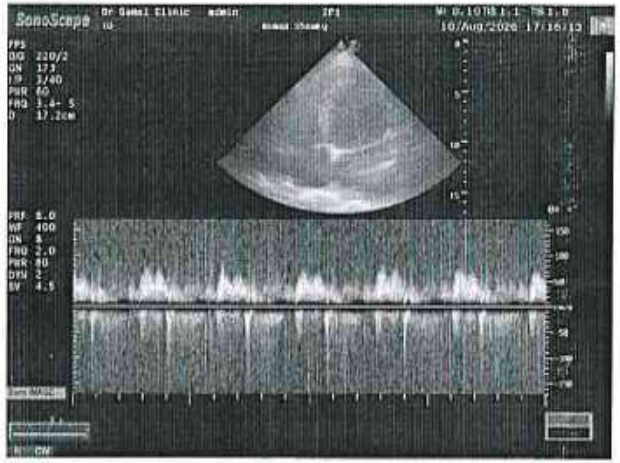

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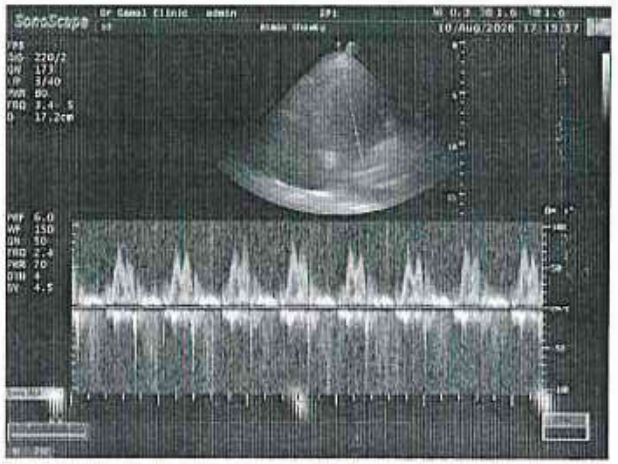

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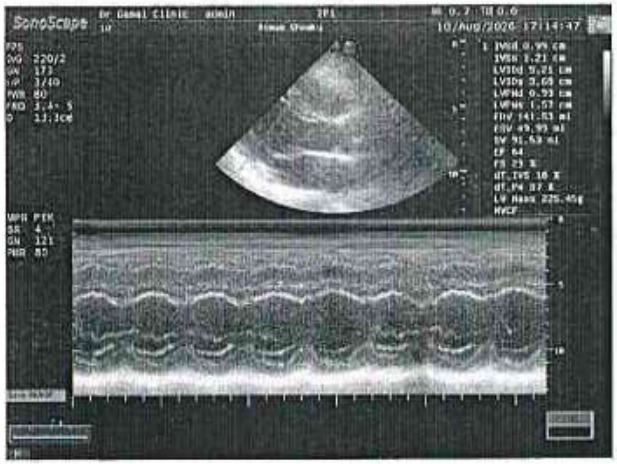
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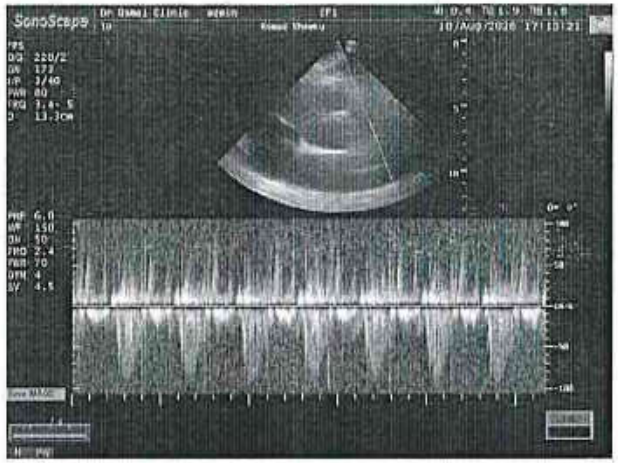
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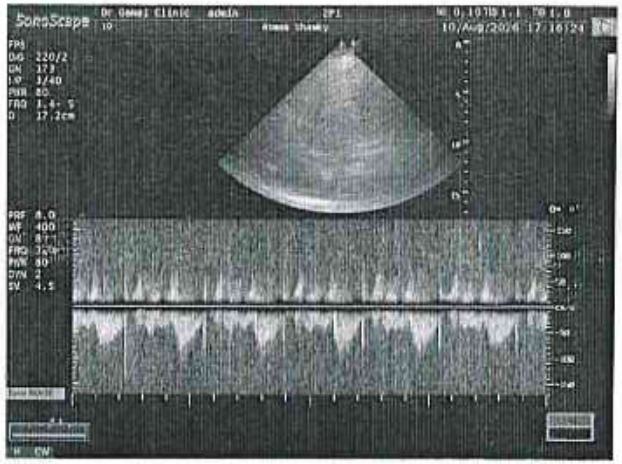
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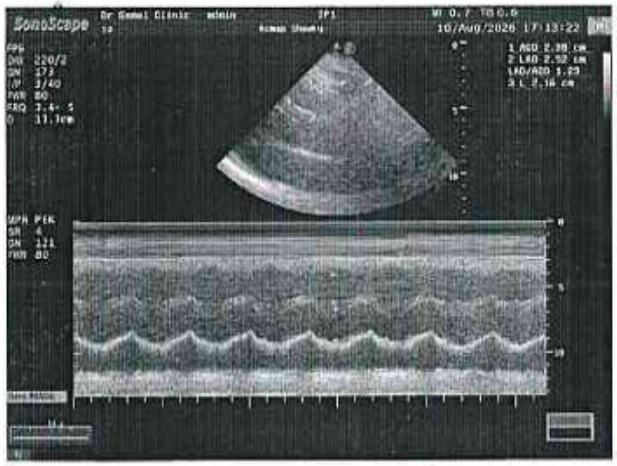
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Cardiologist

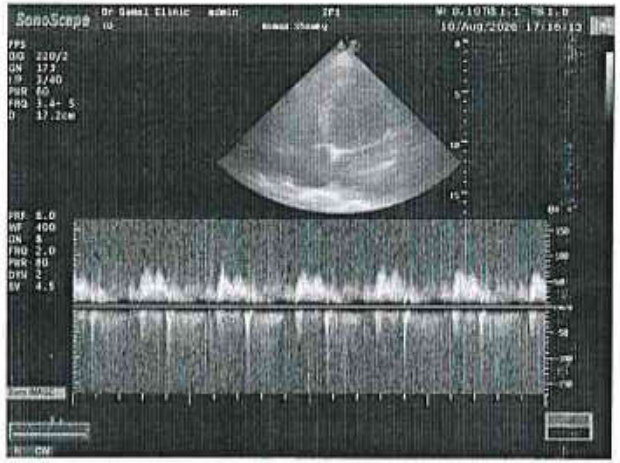

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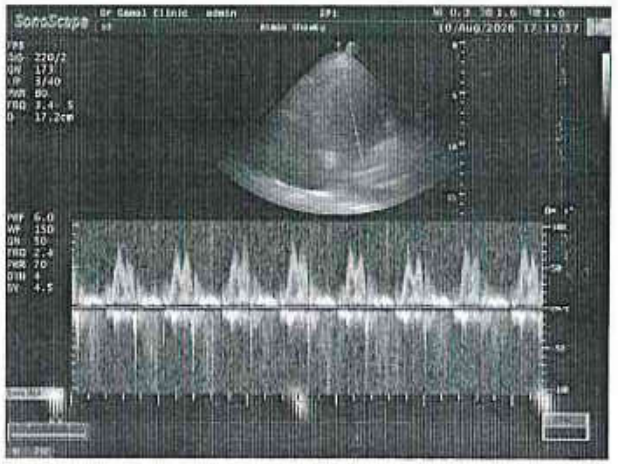

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Cardiologist



اسماء
شوقى عبدالسلام عيسى
ش صلاح الدين-بجوار مدرسة المساعى
مركز اشمون - المنوفية
٢٨: ٠١ ١٠ ١٧ ٠١٠ ٨٥
١٩٨٤/٠١
LC1967251


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٢٠٢٥/٠٤
حاصله على دبلوم تجارة
انثى
مسلمة
متزوجة
مجدى فواد عجمى عجمى شعير
١٢
البطاقة سارية حتى ٢٠٣٢/٠٤/١١

جوستاف روسي
الترفاشونال مصر

إسم المريض ثلاثي: .....

رقم الملف: .....

تقريـــر طبـــي

السيد/.....

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إيماء للطلب المقدم من ..... بشان تقرير طبي للمريض: ا.....

بقسم.....رقم طبي .....

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وبعد الكشف والفحوصات والمتابعة وجد انه:

المريض تعاني مسورم غير محدد بالثدي وكان
اي استكمال علاج موجه فيصبح
صبح اشعة ٦ جرعات قبل الجراحة
واشعة ٩ جرعات بعد الجراحة
وعلاج الي استكمال جرعتين بعد
مقارنة الفحص والاستكمال ١٧ جرعة

تحريرا في

الوقت: .....

التاريخ : ١٣/١٠/٢٠٢٦

الطبيب المعالج

.....

يعتمد ،،

Non-HIO Letter
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